

ADMISSION NOTE - Patient synth_04 - Acute pyelonephritis

PATIENT: Synth Patient 04

ADMIT DATE: [DATE]

CHIEF COMPLAINT: "Fever and back pain for 2 days."

HISTORY OF PRESENT ILLNESS: This is a 32-year-old female with a known history of recurrent UTIs who presents with a 2-day history of progressively worsening right-sided flank pain, subjective fevers (Tmax 102.5F at home), rigors, nausea, and one episode of non-bilious, non-bloody emesis this morning. She also reports associated dysuria, urinary urgency, and frequency which began approximately 4 days ago. She initially attempted to manage these lower urinary tract symptoms with increased water intake without success. The pain is described as a constant, sharp ache in her right flank, rated 8/10 in severity, which radiates towards her groin. She denies gross hematuria, abdominal pain, diarrhea, or constipation.

PAST MEDICAL HISTORY: Recurrent cystitis (approx. 3-4 episodes per year). Seasonal allergic rhinitis.

PAST SURGICAL HISTORY: None.

MEDICATIONS: Loratadine 10 mg PO daily. No known drug allergies.

SOCIAL HISTORY: She is a high school teacher. Denies tobacco use, drinks alcohol socially (1-2 glasses of wine per week), denies illicit drug use. Sexually active with a single male partner, uses oral contraceptive pills.

REVIEW OF SYSTEMS: As per HPI. Otherwise negative except for mild generalized weakness and myalgias. Denies chest pain, shortness of breath, headache, or rash.

PHYSICAL EXAMINATION:

VITALS: T 102.8 F, HR 115 bpm, BP 105/65 mmHg, RR 18, SpO2 98% on room air.

GENERAL: Ill-appearing female, shivering in bed.

HEENT: Mucous membranes are dry.

CV: Tachycardic but regular rhythm, S1/S2 audible, no murmurs.

PULM: Lungs clear to auscultation bilaterally.

ABD: Soft, normoactive bowel sounds. Mild tenderness to palpation in the right upper quadrant. No guarding or rebound. Significant right costovertebral angle (CVA) tenderness elicited with light percussion.

EXT: No peripheral edema.

INITIAL LABS: CBC, CMP, UA with reflex to culture, Blood cultures x2, Lactate ordered.

ASSESSMENT & PLAN:

1. Sepsis secondary to Acute Pyelonephritis: Patient meets SIRS criteria (fever, tachycardia) with a clear infectious source. The primary concern is urosepsis, likely from a gram-negative organism. Plan includes aggressive IV hydration with Lactated Ringer's, empiric broad-spectrum antibiotics with IV Ceftriaxone 1g after blood and urine cultures are obtained. Acetaminophen for fever control.
2. Possible Cholecystitis: The finding of RUQ tenderness is somewhat atypical but noted. LFTs are pending. If pain localizes here or LFTs become elevated, will obtain a RUQ ultrasound. For now, will observe.
3. Nausea/Vomiting: Likely secondary to systemic inflammatory response. Will provide IV Ondansetron as needed.
4. Home Medications: Will continue Loratadine 10mg daily.

PROGRESS NOTE ? Day 1 - Patient synth_04 - Acute pyelonephritis

PATIENT: Synth Patient 04

HOSPITAL DAY: 2

SUBJECTIVE: Patient states she is "feeling a little better." Reports her flank pain has improved from 8/10 to a 4/10. Her nausea has resolved and she has had no further episodes of emesis. She was able to tolerate sips of water overnight. Still feels generally weak and fatigued.

OBJECTIVE:

VITALS: T 100.2 F, HR 95, BP 110/70. Last temp > 6 hours ago. I/O over past 24h: 3.2L in / 2.6L out.

EXAM: Appears more comfortable. CVA tenderness on the right is present but significantly improved. Abdomen is soft, non-tender, RUQ tenderness has resolved. Bowel sounds are present.

LABS: WBC is down to 15.2 from 18.5 on admission. Creatinine stable at 0.9. UA from admission showed >100 WBC, +leuk esterase, +nitrite. Blood cultures show no growth thus far. Urine culture is pending.

ASSESSMENT & PLAN:

1. Acute Pyelonephritis/Sepsis: Good clinical response to IV Ceftriaxone and IV fluids. Inflammatory markers are trending down, and she is now afebrile. Pain is better controlled.
2. Cholecystitis: Now felt to be very unlikely given resolution of RUQ tenderness and normal LFTs on admission panel. No further workup indicated at this time.
3. Disposition: Plan is to continue IV Ceftriaxone for at least another 24 hours. Encourage PO fluid intake as tolerated. If she remains afebrile and is able to tolerate a regular diet, we can plan for transition to PO antibiotics and discharge tomorrow.

PROGRESS NOTE ? Day 2 - Patient synth_04 - Acute pyelonephritis

PATIENT: Synth Patient 04

HOSPITAL DAY: 3

SUBJECTIVE: Patient reports feeling "much better" and is eager to go home. Denies any flank pain, fever, chills, or nausea. She ate her full breakfast without issue. Ambulating in the hallway with no difficulty.

OBJECTIVE:

VITALS: T 98.6 F, HR 82, BP 115/75. Afebrile for >24 hours.

EXAM: Well-appearing, in no distress. Abdominal exam is benign. No CVA tenderness.

LABS: WBC further down to 11.1, Creatinine improved to 0.8. Blood cultures are final with No Growth. Urine culture has a preliminary result of >100,000 cfu/mL of E. coli. Full sensitivities are still pending.

ASSESSMENT & PLAN:

1. Acute Pyelonephritis: Clinically resolved. Patient has met all criteria for discharge: afebrile for >24h, resolution of symptoms, tolerating PO diet and fluids. Ready to be transitioned to oral antibiotics.
2. Disposition: Discharge today. Based on the preliminary urine culture and local antibiogram, she will be prescribed a 7-day course of Ciprofloxacin. Counseled on the importance of completing the full course. Discharge paperwork prepared. Strict return precautions were provided. Will call patient with final urine culture sensitivity results in 2-3 days and change antibiotic if necessary.

LABORATORY REPORT - Patient synth_04 - Acute pyelonephritis

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--- CHEMISTRY PANEL (Admission) ---

Sodium: 137 mEq/L (Ref: 136-145)

Potassium: 3.9 mEq/L (Ref: 3.5-5.1)

Chloride: 101 mEq/L (Ref: 98-107)

CO2: 24 mEq/L (Ref: 22-30)

BUN: 18 mg/dL (Ref: 7-20)

Creatinine: 0.9 mg/dL (Ref: 0.6-1.2)

Glucose: 105 mg/dL (Ref: 70-110)

ALT: 25 U/L (Ref: 7-56)

AST: 22 U/L (Ref: 10-40)

--- HEMATOLOGY (Admission) ---

WBC: 18.5 k/uL (Ref: 4.5-11.0) HIGH

Hgb: 13.1 g/dL (Ref: 12.0-16.0)

Plt: 250 k/uL (Ref: 150-450)

--- URINALYSIS (Admission) ---

Appearance: Cloudy

Leukocyte Esterase: Positive (Large)

Nitrite: Positive

WBCs: >100 /hpf

RBCs: 2-5 /hpf

Bacteria: Many

--- MICROBIOLOGY ---

Blood Culture Bottle 1 of 2: No Growth at 48 hours.

Blood Culture Bottle 2 of 2: No Growth at 48 hours.

Urine Culture: >100,000 cfu/mL Escherichia coli isolated.

Urine Sensitivities: PENDING.

MEDICATION RECORD - Patient synth_04 - Acute pyelonephritis

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--- Home Medications on Admission ---

1. Loratadine 10 mg PO daily

--- Inpatient Medication Administration Record ---

- Ceftriaxone 1 gram IV piggyback every 24 hours
- Acetaminophen 650 mg tablet PO every 6 hours as needed for fever/pain
- Ondansetron 4 mg IV push every 8 hours as needed for nausea
- Lactated Ringer's IV at 125 mL/hr for 24 hours, then saline locked
- Loratadine 10 mg tablet PO daily

--- Discharge Medications ---

1. Ciprofloxacin 500 mg tablet. Take one tablet by mouth two times per day for 7 days.
2. Acetaminophen 650 mg tablet. Take one tablet by mouth every 6 hours as needed for pain.

DISCHARGE ADVICE - Patient synth_04 - Acute pyelonephritis

PATIENT: Synth Patient 04

DISCHARGE DATE: [DATE+2]

DISCHARGE DIAGNOSES:

1. Acute Pyelonephritis due to E. coli
2. Sepsis, resolved
3. Cholecystitis, resolved

BRIEF HOSPITAL COURSE:

Ms. Patient is a 32-year-old female who was admitted to the hospital for acute pyelonephritis and sepsis. On presentation, she had a high fever, right flank pain, and nausea. She was treated with intravenous fluids and antibiotics (Ceftriaxone) and showed significant clinical improvement. Her fever, pain, and other symptoms have resolved. She has been transitioned to oral antibiotics and is now stable for discharge.

DISCHARGE CONDITION: Stable.

DISCHARGE MEDICATIONS:

- Ciprofloxacin 500 mg tablets: Take one tablet by mouth twice a day. You have been given a 7-day supply. It is very important to finish all of the antibiotics.
- Acetaminophen (Tylenol) 650 mg tablets: You may take one tablet by mouth every 6 hours as needed for any residual discomfort.

FOLLOW-UP:

- Please schedule an appointment to see your primary care provider, Dr. Smith, within the next 1-2 weeks for a post-hospitalization check-up.
- A member of our team will call you in the next 2-3 days with the final results of your urine test (sensitivities). If the prescribed antibiotic is not the best one for the bacteria, we will call in a new prescription for you.

PENDING RESULTS AT DISCHARGE:

- Urine culture sensitivities are still pending.

RETURN INSTRUCTIONS:

Please return to the Emergency Department immediately if you experience a return of fever (over 100.4 F), worsening back or abdominal pain, persistent nausea or vomiting, or if you develop a rash or other concerning new symptoms.